



Name	Mrs.RAHILA	Sample Received	07-12-2024 11:40 AM	Sample Collected At	BGSB
Age/Gender	30 Y / Female	Sample Processed	07-12-2024 11:41 AM	Referred By	BGSB
UID	HLD0712202445433	Approved Date	07-12-2024 03:42 PM		

IMMUNOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
*ULTRASENSITIVE TSH	9.546	μIU/ml	0.35 - 4.78
Method: C.L.I.A			

Remarks:

The biological reference range pertains to the age and sex of the Patient.

Note: As the reports are computer generated, it must be treated as an original.**Dr. Anuja Sharma**
Consultant Pathologist
Reg.No. 455**A Unit Of Hexamed Healthcare Services LLP****Corporate Office:** Flat No 203, 2nd Floor, Sagacity Heights, Plot No 27, Kavuri Hills Rd, CBI Colony, Madhapur, Telangana 500033**Jammu Address:** 25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

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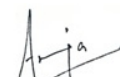
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BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
*GLUCOSE FASTING	97.9	mg/dl	70.0 - 110.0
Method: Spectrophotometry, Hexokinase			

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HAEMATOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
*HEMOGLOBIN	11.73	g/dl	12.0 - 15.0
Method: Cyan-methemoglobin			
*PCV	35.8	%	36.0 - 46.0
Method: Mathematical Calculatio			
*RBC COUNT	4.64	million/ μ l	4.5 - 5.5
Method: Light scatter			
MCV	77.1	fL	83.0 - 101.0
Method: Mathematical Calculation			
MCH	25.3	pg	27.0 - 32.0
Method: Mathematical Calculation			
MCHC	32.8	g/dl	31.0 - 34.5
Method: Mathematical Calculation			
R.D.W.	25.9	%	11.5 - 14.5
Method: Measured, RBC Histogram			
*PLATELET COUNT	159	1000 cells/ μ l	150.0 - 410.0
Method: Light scatter			
MPV	10.87	fL	6.8 - 10.9
Method: Measured Platelet Histogram			
*TOTAL LEUCOCYTE COUNT (TLC)	8.92	1000cells/ μ L	4.0 - 10.0
Method: Peroxidase/Basophil/Lobularity			
*NEUTROPHILS	52.0	%	40.0 - 75.0

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HAEMATOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE NEUTROPHIL COUNT (ANC)	4.64	1000/ μ l	2.0 - 7.0
Method: Peroxidase (Flowcytometry)			
*LYMPHOCYTES	37.0	%	20.0 - 45.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE LYMPHOCYTE COUNT(ALC)	3.3	1000/ μ l	1.0 - 3.0
Method: Peroxidase (Flowcytometry)			
*MONOCYTES	6.0	%	1.0 - 10.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE MONOCYTE COUNT(AMC)	0.54	1000/ μ l	0.2 - 1.0
Method: Peroxidase (Flowcytometry)			
*EOSINOPHILS	5.0	%	1.0 - 6.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE EOSINOPHIL COUNT(AEC)	0.45	1000/ μ l	0.02 - 0.5
Method: Peroxidase (Flowcytometry)			
*BASOPHILS	0.0	%	0.0 - 1.0
Method: Basophil/Lobularity(Flowcytometry)			
*ABSOLUTE BASOPHIL COUNT (ABC)	0.0	1000/ μ l	0.0 - 0.1
Method: Basophil/Lobularity(Flowcytometry)			

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LABORATORY REPORT

*HBA1C, GLYCATED HEMOGLOBIN

RESULT : 4.8 %

Interpretation

HbA1c result is suggestive of non diabetic adults (≥ 18 years)/ well controlled Diabetes in a known Diabetic.

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HBA1C RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of e.g., carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c

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